

Griseofulvin and Antifungal Therapy for Tinea Capitis

Shake liquid before administering. Nausea, vomiting, gastrointestinal disturbances, leukopenia, and diaphoresis may occur.

Patients with excessive losses (e.g., burns) or impaired absorption may require higher doses.

Abbreviations:

Caps = capsules; FDA = U.S. Food and Drug Administration; Fe = iron; G6PD = glucose-6-phosphate dehydrogenase; GI = gastrointestinal; HSV = herpes simplex virus; max. = maximum; Susp = suspension; Tabs = tablets.

> **Note:** 5 mL = 1 teaspoon.

The drug information and doses listed herein are provided as guidelines only and are not comprehensive. Drug selection and dosage should be individualized. Information may have changed since publication. Parenteral and neonatal indications and dosages are not included.

Antifungal Therapy for Tinea Capitis

Griseofulvin remains the only FDA-approved drug for the treatment of tinea capitis in children.

- **Dosing:**
- **Liquid formulation:** 20 mg/kg/day
- **Ultramicrosized tablets:** 15 mg/kg/day
- **Duration:** 6 to 8 weeks
- **Status:** First-line therapy

The use of **fluconazole**, **terbinafine**, and **itraconazole** for pediatric tinea capitis is currently off-label and considered experimental, though evidence

suggests efficacy and good safety profiles. These agents are considered second-line options, and monitoring is recommended when used.

In a large multicenter, double-blinded, randomized trial involving 880 children, **fluconazole** was not found to be highly effective for pediatric tinea capitis.[56] Further studies are needed to establish optimal dosing and long-term safety for these alternative antifungals.

Child Abuse

In the United States in 2003, approximately 906,000 children were identified as victims of child abuse and neglect by child protective services, with an estimated 1,500 deaths attributed to abuse.

Dermatologic findings are present in up to **90%** of abused children and may provide critical diagnostic clues.

Child abuse is further discussed in Chapter 105.

Key References

The full reference list for all chapters is available at www.digm7.com.

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